

SDP Financial Management Services — Co-Employer (Code 316)

Start-Up Cost & Revenue Model | English-language rates | Prepared June 2026

Assumptions

- Lean, remote, founder-led operation built around the **Co-Employer model (Service Code 316)**; able to add clinical/nursing as participant-directed services.
- FMS fees are **paid by the Regional Center outside** the participant's individual budget — they do not reduce participant services, and the fee tier is driven by the **number of providers**, not their wages.
- Employer burden (FICA, Medicare, FUTA, SUI, and **workers' compensation**) is a **funded** pass-through per the DDS approved employer-burden schedule — not founder overhead.
- Surety bond = **20% of the total of all participant individual budgets served**; premium estimated at ~1.5% of the bond face (good credit).
- Average participant individual budget assumed at **\$40,000** for bond sizing (SDP range ~\$15,000–\$50,000+).

1. SDP Co-Employer FMS Rates (English-language, RC-paid outside budget)

Providers in spending plan	Maximum monthly rate
0–4 providers	\$420
5–10 providers	\$660
11+ providers	\$840

Effective May 1, 2023 schedule (English maximums). Non-English maximum (\$925) excluded. Confirm any one-time start-up fee with your vendoring Regional Center.

2. One-Time Start-Up Costs

Item	Low	Expected	High
Entity formation (LLC, registered agent, operating agreement)	\$500	\$1,500	\$2,500
Legal — FMS agreements, Title 17 compliance review	\$3,000	\$7,500	\$15,000
IRS / EDD employer-agent setup (Form 2678, accounts)	\$500	\$1,000	\$2,000
FMS / payroll software implementation & onboarding	\$2,000	\$6,000	\$15,000
Accounting system setup	\$500	\$1,000	\$2,000
Disaster-recovery / business-continuity plan + policies	\$1,000	\$2,500	\$5,000
Website / branding / launch marketing	\$1,000	\$3,000	\$6,000
Equipment / home office	\$1,000	\$2,000	\$4,000
Subtotal (core, SOC deferred)	\$9,500	\$24,500	\$51,500

Optional SOC 1 audit (often deferred to Year 2): readiness \$8,000–\$20,000; Type I \$10,000–\$25,000. With SOC, one-time subtotal ~\$27,500 / \$51,500 / \$96,500.

3. Year-1 Recurring Operating Costs

Item	Low	Expected	High
Surety bond premium (see Section 4)	\$1,200	\$3,000	\$6,000
Own insurance — GL + E&O + cyber + fidelity + employer liability	\$2,500	\$4,500	\$8,000
FMS / payroll software subscription	\$4,000	\$9,000	\$18,000

Outsourced bookkeeping / CPA / payroll-tax filing	\$6,000	\$12,000	\$20,000
CA franchise tax	\$800	\$800	\$800
Bank fees, phone, misc. software	\$2,000	\$3,500	\$5,000
Subtotal (before staff labor)	\$16,500	\$32,800	\$57,800

Staff labor (payroll-capable, 1–2 FTE): \$0 (founder unpaid) / \$80,000 / \$135,000. Worker wages and funded employer burden are pass-through, not shown here.

4. Surety Bond (scales with participants served)

Participants	Total budgets (~\$40k ea.)	Bond face (20%)	Annual premium (~1.5%)
10	\$400,000	\$80,000	~\$1,200
25	\$1,000,000	\$200,000	~\$3,000
50	\$2,000,000	\$400,000	~\$6,000
100	\$4,000,000	\$800,000	~\$12,000

Annual bond review/renewal applies once served budgets total \$500,000 or more in a state fiscal year (~13+ participants).

5. Revenue Projection (Co-Employer, blended ~\$525/participant/month)

Participants	Annual FMS revenue (RC-paid)
10	~\$63,000
25	~\$157,500
50	~\$315,000
100	~\$630,000

6. Cash Need & Break-Even Scenarios

Scenario	Year-1 cash need	Break-even
Lean solo (founder unpaid, SOC deferred, ~15 participants)	~\$50,000–\$65,000	~10–15 participants
Staffed (1.5 FTE, SOC readiness + Type I, ~25 participants)	~\$160,000–\$215,000	~30–40 participants

Plan 3–6 months of runway before first billing (entity setup + 45-day RC vendorization review + onboarding), plus working capital to float payroll between paying workers and RC reimbursement.

7. Adding Nursing / Clinical Services under Co-Employer

- **Permitted & fundable.** SDP allows participant-directed nursing; the spending plan must specify a qualified provider (RN, or LVN/LPN under RN supervision, licensed in California).
- **Generic-resources-first.** The participant must exhaust Medi-Cal and other generic nursing resources before SDP-funded nursing — this limits how much nursing flows through you.
- **Fee is wage-decoupled.** A nurse counts as one provider; your tier (\$420/\$660/\$840) is unchanged by the nurse's higher wage.
- **Workers' comp is funded.** The higher home-health class-8835 premium (CA ~\$2–\$4 per \$100 of payroll vs. ~\$0.30–\$0.50 clerical) is part of the funded employer burden, not your overhead.
- **Your real exposure.** The experience modifier on your master WC policy (a clinical claim raises your renewal across all participants), plus higher employer / professional / abuse-&-molestation liability — budget toward the high end of own-insurance, and confirm carrier appetite for clinical exposure.
- **Verify with the RC.** Confirm they will authorize/fund nursing in your participants' spending plans and accept the FMS as employer of record for skilled (vs. supportive) nursing.

Disclaimer. Planning-grade estimates compiled from public DDS, WCIRB, and market sources for internal evaluation only — not legal, tax, financial, or insurance advice, and not a quote. Verify all FMS rates, bond, insurance, audit, and employer-burden requirements with your vendoring Regional Center, DDS, a licensed broker, and a CPA before relying on these figures.

Sources: DDS SDP FMS Revised Rates; DDS Summary of Approved FMS Employer Burden (2025); DDS SDP FAQ (nursing qualifications, generic-resources-first); DDS FMS Models Comparison Chart; Title 17 CCR §§54310, 54327, 58884, 58886, 58887; DDS Directive D-2025-SDP-001 (Additional Requirements for Entities); WCIRB California classification 8835; CA Dept. of Insurance WC rate comparison; surety bond and SOC audit market cost guides.